



May 19th, 2026

Dr. Michelle Bonapace-Potvin
University of Minnesota Department of Surgery
11-132 Phillips-Wangensteen Bldg.
516 Delaware Street SE
Minneapolis, MN 55455

Letter of support for chest surgery
RE: Molly (Margaret) Froman; DOB: 02/14/1996

Dr. Michelle Bonapace-Potvin,

I am writing this letter of support for chest surgery at the request of Molly (Margaret) Froman. I have been seeing them for weekly individual therapy since August 2025. I first began seeing them to address concerns around gender identity, and related mental health struggles. I last met with Molly on November 7th, 2025 to assess their capacity to provide informed consent for chest surgery.

Molly currently appears to meet criteria for F64.0 Gender Dysphoria. They have exhibited persistent and long-standing gender and anatomic dysphoria related to their birth assigned sex. Other possible causes of apparent gender incongruence have been identified and excluded making Gender Dysphoria the appropriate diagnosis for this client. They currently appear to meet the World Professional Association for Transgender Health (WPATH) Standards of Care for surgery of gender dysphoric persons.

In regard to gender identification, Molly reported that they currently identify as transgender and non-binary. They reported their first recall having discomfort regarding their gender around 10 years old when they began dressing in clothing from the boys section and noticing improved emotional comfort when doing so. They reported notable distress during puberty and memories of hiding the physical signs due to fear of being perceived and identified as a girl. They reportedly came out to others as transgender at age 26 and have remained consistent in this identity since that time.

As described, Molly reported that they began experiencing chest dysphoria since puberty and have consistently felt uncomfortable with their chest, specifically recalling discomfort when receiving any external attention directed towards their chest. They reported significant distress regarding sensations of having excess breast tissue, particularly in that it may be perceived as feminine and lead to misgendering. They also reported body dysphoria related to menstruation. They stated that they are seeking chest surgery to alleviate chest related dysphoria that cannot be otherwise managed by other medical or psychological interventions.

Molly meets criteria for F34.1 Persistent Depressive Disorder; in partial remission, F42.2 Obsessive-Compulsive Disorder, and F41.1 Generalized Anxiety Disorder. Their mental health symptoms appear to be reasonably well controlled. They appear to have a history of being proactive in managing their mental health as evidenced by seeking out mental health services, staying consistent with medication management, and maintaining a strong social support network. They report a plan to continue individual therapy with this writer to aid in managing their symptoms post surgery. As a result, there currently appear to be no unaddressed mental health concerns that would present need for caution in moving forward with their gender affirming goals. Furthermore, this procedure is expected to improve their overall sense of mental and physical well-being.

Molly has engaged in dialogue with this writer about the potential costs and benefits of having chest surgery and appears aware of potential risks. They appear able to provide informed consent to this procedure. They report having a plan for care following surgery and during recovery. Molly appears to have a realistic expectation of the recovery process.

Given the aforementioned information, I support Molly's decision to undergo chest surgery at this time, as it appears medically necessary in managing gender dysphoria. In some cases, denial of access to gender affirming surgeries can lead to increased isolation, decreased functional performance or interpersonal functioning, and exacerbated mental health symptoms. Surgery is expected to increase comfort with self, as well as improve interpersonal, sexual, and overall functioning.

This writer's support is based on the psychological indication for surgery and did not include a physical examination to assess medical contraindications for the surgical procedure. This letter of support is valid for one year from the date of approval. If you have any questions, or are in need of additional information, I am available for coordination of care. Please do not hesitate to contact me at 507-9930675 or via email; alley@indigocounselingcenter.com

Sincerely,

A handwritten signature in cursive script that reads "Alley Maki".

Alley Maki, MA, LPCC (she/her)